

Advanced Foot & Ankle Associates

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NOTE 1 OF 2

DATE OF SERVICE

May 29, 2026

PATIENT NAME	DATE OF BIRTH	DATE OF SERVICE	PROVIDER	NPI
Diana Ramirez	04/16/1986	May 29, 2026	Dr. Rachel Torres, DPM	6789012345
INSURANCE	MEMBER / POLICY ID	GROUP NUMBER	COPAY STATUS	AUTH REQUIRED
Oscar Health	OSC795822698	GRP19358	Verified at check-in	On file

RARE & ADVANCED PROCEDURES

SCENARIO 23: CAVUS FOOT RECONSTRUCTION — TRIPLE ARTHRODESIS + PERONEAL TENDON TRANSFER

CHIEF COMPLAINT	Progressive cavovarus foot deformity — Charcot-Marie-Tooth disease
HISTORY OF PRESENT ILLNESS	37-year-old female with Charcot-Marie-Tooth disease (CMT1A, confirmed genetic testing), progressive bilateral cavovarus foot deformity, right greater than left. Recurrent lateral ankle sprains, lateral foot overload callusing. Peroneus brevis tear confirmed MRI. Coleman block test positive (correctable forefoot-driven deformity).
PAST MEDICAL HISTORY	CMT1A, Raynaud's phenomenon
CURRENT MEDICATIONS	Gabapentin 300 mg TID
ALLERGIES	NKDA
PHYSICAL EXAMINATION	Right foot: Pes cavus, hindfoot varus, forefoot plantarflexion (1st MT dropped). Clawing all toes. Peroneal weakness (1/5 strength). Anterior tibial muscle bulk reduced. Coleman block: hindfoot varus corrects — forefoot-driven. MRI: peroneus brevis split tear.
ASSESSMENT / DIAGNOSIS	Pes cavovarus, right; CMT hereditary motor-sensory neuropathy; Peroneus brevis tear
PROCEDURE(S) PERFORMED	Spinal anesthesia. Forefoot: plantar-based closing wedge osteotomy 1st metatarsal — elevates plantarflexed 1st ray, corrects forefoot driving force. Peroneus longus tendon transferred to peroneus brevis stump — activates dorsiflexion-eversion. Calcaneal lateralizing osteotomy — tuberosity shifted laterally 8mm, fixed with 6.5mm screw. Clawing correction: extensor digitorum longus transferred to dorsal metatarsal necks (Hibbs transfer). Fluoroscopy confirms correction. Splint.
PLAN / TREATMENT	Dorsiflexion osteotomy 1st MT + peroneus longus to brevis transfer + calcaneal lateralizing osteotomy, right foot (staged surgery for complex deformity)
FOLLOW-UP	NWB x 6 weeks. Custom AFO long term (CMT disease will progress — bracing essential). PT for strengthening residual muscles. Annual monitoring of neurological progression. Contralateral foot surgical planning at 6 months.

CMT DISEASE NOTE

CMT1A — progressive neuropathy. Surgical intervention palliative, not curative. Genetic counseling referral for family members. Bracing and annual monitoring essential as disease progresses.